



LGBTIQ+ Inclusivity for Patients Policy

Scope

Site	Department, division, or operational area	Applicable to
Royal Perth Bentley Group (RPBG)	All areas	All staff

- RPBG recognises that every person that identifies as Lesbian, Gay, Bisexual, Transgender, Intersex and Queer Plus (LGBTIQ+) has terms and language they prefer when describing their own sex, gender, sexual orientation, and variations in sex characteristics. The use of this acronym is not intended to be limiting or exclusive of certain groups and RPBG recognises that not all people will identify with this acronym or use these specific terms and that the fluidity of language and terms change and evolve over time, including the acronym
- LGBTIQ+ will be used throughout this document in alignment with the peak body [LGBTIQ+ Health Australia](#)

Purpose

This policy assists clinicians to provide holistic treatment of LGBTIQ+ consumers while in the care of RPBG.

Links within document

- [Appendix I: RPBG LGBTIQ+ flyer](#)
- [Appendix II: Best practices for a gender diverse affirming environment](#)
- [Appendix III: Mental Health Division Name and Pronouns Purple Sticker](#)
- [Appendix IV: Additional LGBTIQ+, sexuality, gender and bodily diversity definitions](#)

Policy directive

- For the purposes of this document, the patient, consumer, or client will be referred to as the “consumer”
- All consumers are entitled to be free from victimisation, re-traumatisation, violence and sexual assault regardless of their age, gender, origin and sexuality
- Staff are to maintain sensitivity and openness regarding sexual orientation and LGBTIQ+ identity
- RPBG will ensure the physical environment is appropriate to deliver and facilitate safe and effective care that encompasses the LGBTIQ+ consumer related needs.

Considerations when caring for LGBTIQ+ consumers

- Consumers who identify as LGBTIQ+ will be provided a nurturing and empathic care pathway with consideration to the below:
Many LGBTIQ+ people are adversely affected by multilayered discrimination, marginalisation, and stigma. Risk factors for their mental health include violence, bullying or rejection and discrimination from school, family, friends, and workplaces and from society more generally. Risk factors for intersex people can also include rejection and harassment, being forced to conform to gender norms, or pain and scarring from childhood genital surgeries or forced hormone use.¹
- Mental health issues should not be directly associated to a consumer's identified LGBTIQ+ status and must be assessed independently
- Consideration must be given regarding use of language with LGBTIQ+ consumers. Staff are not to assume they know a consumer's gender and/or sexuality and/or bodily diversity based on their presentation
- RPBG supports the **International Convention on the Elimination of All Forms of Racial Discrimination**.

The multidisciplinary team (MDT) will

Across all RPBG services (i.e., general health and mental health)

- Consider that LGBTIQ+ consumers may
 - Be reluctant to share information regarding sexual practices and hormone use
 - Have prior negative/ traumatic experiences with clinicians or within the health care setting
 - Have experienced insensitivity due to engaging in relationships with multiple partners or individuals of the same sex
 - Be particularly vulnerable in regard to their sexual safety. Refer to **RPBG Sexual Safety Policy**
 - Be vulnerable to the possibility of anxiety, depression, and suicide ideation.
- Display LGBTIQ+ welcoming signage in high visibility areas and ensure that resources are available for those consumers and/or significant other(s), personal support persons (PSP) looking for further LGBTIQ+ information and/or supports, e.g., LGBTIQ+ Allies
- When obtaining consumer information there may be privacy concerns. Ensure consumers are provided an opportunity to identify where and/or how they wish to discuss sensitive information (e.g., in front of family or PSP, in a shared room, in private or interview area)
- Establish comfort and trust. Some consumers may not be comfortable answering questions until a level of trust has been built. Consumers need to be reassured that they do not need to answer if they are not comfortable and that they can change their details at any time in the future.

Considerations when caring for LGBTIQ+ consumers – The multidisciplinary team (MDT) cont'd

It is essential for staff asking a consumer their sex recorded at birth to also collect information regarding the consumer's gender and to refer to that person by the recorded gender and pronouns. Referring to a person by the sex recorded for them at birth can be an uncomfortable or even, traumatic, and harmful experience for trans consumers.

If staff do need to collect this information for diagnostic purposes, it is important to not assume experiences, body parts, or other characteristics based on that information. Asking and noting what parts a person has is more than sufficient and helpful for cis and trans people (see [TransHub](#) and [Appendix IV: Additional LGBTIQ+, sexuality, gender and bodily diversity definitions](#)).

- Before asking a consumer about their sexual and/or gender and/or bodily diversity, ask:
 - Permission to broach these topics (e.g., **“Is it okay if I ask some questions about your sexuality?”**; **“The next few questions are about ‘X’; is it alright if I ask those now, or would you like to come back to them later?”**)
 - Leading questions like **“How do you describe your sexuality?”** or **“How would you describe your gender identity?”**
 - **“Is there any specific label or word you'd like us to use when we talk about your gender?”**
- Make it clear that there are no right or wrong answers
- Explain why asking these questions is important to the consumer's care
- All consumers should be offered an opportunity to declare their preferred pronouns
- Consider the consumers' reasons for presenting when making decisions about the depth of information to gather about sexuality, gender and/or bodily diversity
- Consider the facets of medical and non-medical transition or identity actualisation
- Some LGBTIQ+ people think that disclosing information to one staff member means that it will be communicated to all staff providing them service. It may be relevant to ensure that this is not the case. Staff should not disclose the consumers LGBTIQ+ identity to others without their permission, except where relevant in the provision of clinical care. This includes how information will be shared (e.g., gain consent from consumers to document and release information before including LGBTIQ+ identity in discharge summaries to general practitioners) ²
- Be aware that consumers' biological families may not always know and/or be supportive of their relatives. RPBG services' primary obligation is to protect the consumer's needs
- A partner of the same or different sex can be allocated by the patient as their PSP or Next of Kin and should be involved in all relevant aspects of consumer care
- Use the principles of trauma informed care ²

Considerations when caring for LGBTIQ+ consumers – The multidisciplinary team (MDT) cont'd

- Be cognisant that medical consultations, procedures, and diagnostic tests, have the potential to cause the consumer anxiety (e.g., consumers who use binding, etc. may be hesitant or concerned to remove the binder/ other garments)
 - Adhere to and apply the basic principles of respect, explanation, consent, and privacy
 - Consider the presence of a PSP and/or chaperone to support the consumer
 - Ensure **physical examination** is necessary and relevant to the presenting issues, regardless of gender presentation, or identity
 - Ask consumers if they have a preferred term to be used for their body parts
 - Ensure sensitivity and confidentiality when history taking to determine the individualised changes in the context of surgical or medical intervention or use of binding/packing/tucking (refer to *Definitions* section)
 - Are there any key factors that may affect treatment or tests being done? For e.g. surgical or medical interventions, medications
 - **Note:** Consumers who have undergone gender affirming surgeries may have varying physical exam findings depending on the procedures performed and any complications ³
 - Check if assistance is required with undressing, binds, tucks, etc. where relevant and refer to safe binding, packing, and tucking practices (e.g., <https://www.rainbow-project.org/safe-binding-and-packing/>)
- Identify higher-risk consumers and establish or update identified health risks and planned mitigation strategies in the general health and MH, relevant documentation suite
- Seek guidance from clinicians experienced in the treatment/care of LGBTIQ+ consumers and/or other health service providers consumers may be accessing for specific interventions
- Discuss preferred room options with the consumer where applicable and specific access to toilet and bathroom facilities that may be required
- Staff allocation should be considered and based on clinical need, availability and consumer preference. Consider allocating to a LGBTIQ+ Ally if possible
- Provide or offer all consumers who identify as LGBTIQ+ and their significant others or PSP with information about the appropriate feedback systems regarding compliments and complaints
- Consider guidance in [Appendix II: Best practices for a gender diverse-affirming environment](#).

Considerations for mental health specific areas

- Use the *Name and Pronouns Purple Sticker* (see [Appendix III](#)) to indicate the consumer's name and pronoun in the inside cover of the patient's integrated progress notes. Consider the consumer's privacy needs when displaying this sticker and periodically check with the consumer about their pronouns
- Complete the *Personal Support Person and Consent to Release/ Receive Information Form MR001.05*, indicating what information can be shared with each person identified
 - Consider closely the need to send out discharge summaries and/or other documentation to the address of a consumer, as they may not have disclosed any information related to their sexual and/or gender and/or bodily diversity to those they live with. Ensure this information is documented clearly in integrated progress notes, comprehensive care plans, *MH Treatment, Support and Discharge Plan (TSDP)* and other relevant documentation.
- When completing risk assessments
 - Consider the increased risk profile of the LGBTIQ+ population. Refer to **WA Department of Health Triage to Discharge Mental Health Framework for Statewide Standardised Clinical Documentation (SSCD)**
 - Discuss with the consumer any additional needs related to LGBTIQ+ identity and how they might best be managed
 - Consider any history of incidents/allegations or breaches of LGBTIQ+ safety
 - Document findings comprehensively across the MH documentation suite.
- Provide an RPBG LGBTIQ+ flyer (refer to [Appendix I](#)) during admission (placed in admission packs for inpatient areas) and display the LGBTIQ+ feedback poster and collection boxes in reception areas in wards and services (optional for community mental health services).

Professional training

- All new employees are provided with information at Induction regarding EMHS' aims to support and respect diversity. All staff are encouraged to complete the LGBTIQ+ Package in the **EMHS Equity, Diversity, and Inclusion MyLearning Suite**
- The Mental Health Division (MHD) Inpatient service staff are required to complete the **EMHS Equity, Diversity, and Inclusion MyLearning Suite** or the Back to Basics LGBTIQ+ face to face training as part of Rainbow Tick Accreditation compliance
- MHD services will provide additional training and support for identified LGBTIQ+ Rainbow Allies as a face-to-face workshop by the LGBTIQ+ Project Officer. Staff can nominate as a LGBTIQ+ Ally via the [LGBTIQ+ HealthPoint page](#).

Rainbow Tick Accreditation

This consists of 6 standards against which services can be formally accredited to demonstrate LGBTIQ+ inclusive practice and service delivery. RPBG MHD inpatient services' will consider these standards during strategic clinical services planning and aim to comply with these standards in all care delivered to consumers and their PSPs.

Compliance monitoring

Rainbow Tick Service Accreditation process will indicate the MHD's compliance in providing an inclusive service for mental health inpatient services.

Hospital wide compliance with this policy will be monitored during the course of clinical incident investigations via Datix Clinical Incident Management System (CIMS) and complaints via the Consumer Feedback Module.

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Aboriginal Health Impact Statement and Declaration

The voice of the Aboriginal community is reflected in the Policy through consultation with the Aboriginal Health Strategy Team which includes all Aboriginal staff (clinical and non-clinical) across EMHS including the Aboriginal Health Liaison Officers, Aboriginal Health Strategy Team, and Aboriginal Acute Care Coordination Team to ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated (ISD Record 3004).

Note: Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

Related policies, practice standards, clinical guidelines

R PBG Policy Hub

- [Special and Supportive Observations Policy](#)
- [Sexual Safety Policy](#)

EMHS Policy Hub

- [Onboarding Policy](#)
- [Sexual Safety in Mental Health Policy](#)
- [Chaperone Policy](#)

WA Department of Health Policy Hub

- [Triage to Discharge: Mental Health Framework for Statewide Standardised Clinical Documentation \(SSCD\)](#)
- [Lesbian, Gay, Bisexual, Transgender, Intersex Health Strategy 2019 - 2024](#)
- [Responding to an Allegation of Sexual Assault Disclosed within a Public Mental Health Service – Office of the Chief Psychiatrist](#)
- [Reporting Notifiable Incidents – Public Mental Health Services – Office of the Chief Psychiatrist](#)
- [Chief Psychiatrist's Standards for Clinical Care](#)
- [Chief Psychiatrist's Sexual Safety Guidelines](#)

Other

- [WA Equal Opportunity Act 2010](#)
- [WA Gender Reassignment Act 2000](#)
- [WA Mental Health Act 2014](#)
- [Sexual Discrimination Act 1984 \(Amendment August 2013\) – Federal Register of Legislation](#)
- [International Convention on the Elimination of All Forms of Racial Discrimination](#)

Related forms

- *Personal Support Person and Consent to Release/ Receive Information Form MR001.05*
- *Name and Pronouns Purple Sticker*

Links to resources

- [Equity, Diversity and Inclusion MyLearning Suite](#)
- [R PBG Take 5: What is Trauma and Trauma Informed Care?](#)

Related national standards

ACSQHC NSQHS Standards 2nd Edition (2021)

Standard 1: Clinical Governance; Standard 2: Partnering with Consumers; Standard 5: Comprehensive Care Standard; Standard 6: Communicating for Safety

NSMHS 2010: Standard 1: Rights and Responsibilities; Standard 2: Safety; Standard 8: Governance, Leadership and Management

Rainbow Tick Standards

LGBTIQ+ Inclusivity Policy

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References

1. Strauss, P., Hill, N. T. M., Marion, L., Gilbey, D., Waters, Z., Moore, J. K., Costanza, M., Lamblin, M., Robinson, J., Lin, A., Perry, Y. (2022). *Suicide prevention in LGBTQA+ young people: best practice guidelines for clinical and community service providers*. Telethon Kids Institute: Perth, Australia.
2. The Institute on Trauma and Trauma-Informed Care. *What is Trauma Informed Care?* University at Buffalo, Buffalo Centre for Social Research. Accessed 17 Oct 2023. <http://socialwork.buffalo.edu/social-research/institutes-centers/institute-on-trauma-and-trauma-informed-care/what-is-trauma-informed-care.html>
3. Colton L.K and Fitzgerald, K. *Affirmative Psychological Testing and Neurocognitive Assessment with Transgender Adults*. Psychiatric Clinics of North America, Volume 40, Issue 1, 2017, Pages 51-64, ISSN 0193-953X, ISBN 9780323509855, <https://doi.org/10.1016/j.psc.2016.10.011>.
4. LGBT Health Education Center. (2017). *The Providing Affirmative Care for Patients with Non-Binary Gender Identities*. The National LGBT Health Education Center. The Fenway Institute. Boston. Massachusetts.
5. Deutsch, M. (2016). *Guidelines for the Primary and Gender-Affirming Care of Transgender and Gender Nonbinary People*. 2nd edition. San Francisco. USA.
6. LGBTIQ+ communities. Glossary of common terms. CFCA Resource Sheet, Published by the Australian Institute of Family Studies, November 2019. Last updated February 2022. <https://aifs.gov.au/cfca/publications/lgbtiq-communities>

Appendix I: RPBG LGBTIQ+ flyer



Government of Western Australia
East Metropolitan Health Service



Please **share** your **feedback** and **ideas** about **how** we can be **more** **LGBTIQ+** inclusive

Part of our values and mission statement is to recognise and support the diversity of all members of the community we serve.

We are improving the service we provide by ensuring it is LGBTIQ+ inclusive. We welcome all feedback, comments and compliments about your experience with us.

Place your suggestions, thoughts and feedback forms into the box provided. This can be done anonymously.

Alternatively, provide feedback via:

- @ RPBGRainbowTick@health.wa.gov.au
- 92241637
- Feedback form in each area
- Talk to Rainbow Allies



rph.health.wa.gov.au

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Appendix II: Best practices for a gender diverse-affirming environment ⁴

Best Practices

Creating an Affirming Environment for Transgender and Gender Non-Conforming Patients

BEST PRACTICES	EXAMPLES
When addressing patients, avoid using gender-specific terms like “sir” or “ma’am.”	“How may I help you today?”
When talking about patients, avoid pronouns or other gender-specific terms. If you have a record of the name used by the patient, use it in place of pronouns. Never refer to someone as “it.”	“Your patient is here in the waiting room.” “Max is here for a 3 o’clock appointment.”
Politely ask if you are unsure about a patient’s name or pronouns used.	“What name would you like us to use, and what are your pronouns?” “I would like to be respectful—how would you like to be addressed?”
Ask respectfully about names if they do not match in your records.	“Could your chart be under another name?” “What is the name on your insurance?”
Did you goof? Politely apologize.	“I apologize for using the wrong pronoun—I did not mean to disrespect you. How would you like for me to refer to you?”
Only ask information that is necessary for providing care.	Ask yourself: What do I know? What do I need to know? How can I ask in a sensitive way?

Post this sheet on your wall or desk as a helpful reminder!

Appendix III: Mental Health Division Name and Pronouns Purple Sticker

Name
.....
Pronouns.....

Name
See inside file

Appendix IV: Additional LGBTIQ+, sexuality, gender and bodily diversity definitions

Aromantic / Aro	Describes people who do not experience romantic attraction to others and therefore may or may not also identify as asexual (see below). ²
Asexual / Ace	A sexual orientation that reflects little to no sexual attraction, either within or outside relationships. This does not necessarily imply a lack of libido or sex drive. ⁴
Binding	Tight fitting, multiple garments may be used (e.g., bras, shirts, ace bandages, or a specially made <i>binder</i>) to provide a flat chest contour. Some people may experience, restricted breathing, breast pain, local skin irritation, or fungal infections. ⁵
Bisexual	A person of any gender who has romantic and/or sexual relationships with and/or is attracted to people from more than one gender.
Brotherboy	A term used by some Aboriginal and/or Torres Strait Islander trans people who were assigned female at birth but identify as male, encompassing gender and cultural identities. ²
Coming Out	The process of beginning to share one's sexuality or gender with others. This process may also sometimes be referred to as "inviting in" or disclosing one's identity. ²
Cisgender / Cis	Describes people whose gender identity corresponds to the sex they were assigned at birth. ⁵
Cisgenderism	Refer to TransHub .
Cisnormativity	The expectation that sex-assigned at birth matches gender where the only two options are male or female. ²
Deadnaming	Describes the act of intentionally or unintentionally referring to a trans person by the name that they used before affirming their gender. This may also be referred to as "misnaming" and is often experienced as invalidating and distressing. ²
Gay	Men who are sexually/romantically attracted to other men. The term gay can also be used in relation to women who are sexually and romantically attracted to other women.
Gender	Who you know yourself to be. This may be called gender identity, or simply gender (e.g., man/woman/non-binary/male/male/female). Refer to TransHub .

Gender Affirmation / Affirming	An interpersonal, interactive process whereby a person receives social recognition and support for their gender identity and expression. ⁴
Gender Affirming Care	A model of health care that is non-judgemental and respectful of a person's gender identity, and that supports an individual's unique needs and autonomy in relation to their gender. It may encompass psychological and/or peer support, as well as support for medical and/or social transition. ²
Gender Binary	The definition of gender as consisting of either male or female, without experiences outside of or in between these two concepts.
Gender Dysphoria	Subjective mood/affect disturbance experienced by some transgender persons whose gender identity is the opposite of their assigned sex at birth, or, who feel clinically significant distress or impairment due to a noncongruence of their gender role and gender identity. ⁴
Gender Expression / Role	The way in which someone's gender is expressed externally to others, for e.g., through behaviour or clothing choices. Gender expression and gender identity are distinct constructs that may or may not align. ²
Gender Fluid	A person whose gender identity is not fixed, which may change over time or day to day.
Gender Minority	A person who identifies as transgender or gender nonconforming, and/or whose gender identity or expression differs from the conventional gender binary. The LGBTIQ+ is increasingly referred to as sexual and gender minorities. ⁶
Gender Neutral	When societal constructs avoid differentiating or specifying based on gender, including gender neutral language (e.g., pronouns, titles) or spaces (e.g., bathrooms).
Gender Nonconformity	Refers to the extent to which a person's gender identity or expression differs from the cultural norms prescribed for people of a particular sex. ⁶
Heteronormative	The view that heterosexual relationships are the only natural, normal, and legitimate expressions of sexuality and relationships, and that other sexualities or gender identities are unnatural and a threat to society. ⁶
Heterosexuality	An individual who is sexually and/or romantically attracted to the opposite gender. ⁶

Homophobia	Refers to negative beliefs, prejudices and stereotypes that exist about people who are not heterosexual. ⁶
Intersex	People born with natural variations to sex characteristics, including genitalia, other sexual or reproductive parts of the anatomy, hormones, and chromosomes.
Lesbian	A person who self-describes as a woman and who has experiences of romantic, sexual and/or affectional attraction solely or primarily to other people who self-describe as women. ⁵
Misgendering	Where a person is described or addressed using language that does not match their gender identity, e.g., the incorrect use of pronouns or other words that traditionally have gendered applications (e.g., pretty, handsome, etc.). ²
Non-binary	Gender identity that may be expressed as other than woman or man, including gender fluid (see above), gender neutral, androgynous, or not identifying with a gender at all. ²
Packing	Involves the placing of a penile prosthesis in one's underwear, giving both an outward appearance as well as reducing gender dysphoria. ⁵
Pronouns	Words used to refer to people when using a gendered term, e.g., he/him/ his, she/her/hers, they/them/theirs. Some people may use multiple pronoun sets, e.g., she/they or non-gendered pronouns, e.g., xe/xem/xyr, ze/hir/hirs, and ey/em/eir. ²
Queer	A term used by some people to describe non-conforming gender identities and sexual orientations. The meaning of 'queer' has changed from being a slur, to being reclaimed by some LGBTIQ+ people and rejected by others.
Questioning	Describes people who are in the process of exploring their sexuality and/or gender. ²
Sex	A person's biological, physical sex characteristics.
Sexuality / Sexual Orientation	A person's romantic or sexual attraction to others, which may not be related to their gender, or vice versa.

Sister-girl	A term used by some Aboriginal and/or Torres Strait Islander trans people who were classified male at birth but live as women, encompassing traditional cultural female practices. ⁶
Transgender or Trans	People who identify their gender as not matching their sex assigned at birth or may reject the idea of gender altogether. Many people of trans experience identify simply as women or men and may undergo modification to their dress, body and/or legal status. Related pronouns include he, she, they, hu, fae, ey, ze, zir and hir. ^{2, 6}
Transitioning	A range of processes that some trans people may experience in order to affirm their gender and may choose to undergo some, all, or none of these processes, including changing name, pronouns, dress, body (e.g., hormone therapy, gender affirming surgeries) and/or legal status (e.g., changing legal name and sex/gender on government documents). ²
Trauma-Informed Practice / Care	A strengths-based framework based on knowledge and understanding of how trauma affects people's lives, their service needs and embraces a message of hope and optimism that recovery is possible. ³
Tucking	The testicles and penis are moved into the inguinal canal and the penis and scrotum are moved posteriorly in the perineal region to allow a visibly smooth crotch contour. Tight fitting underwear, adhesive, duct tape or a special undergarment known as a gaffe is then worn to maintain this alignment. This practice may cause local skin effects, urinary trauma, infections, or testicular complaints. ⁵